



A SOOTHEMADE BOOK

Caregiving, the First 90 Days

*For the family member who has just had
everything change.*

Soothemade

A SMALL APOTHECARY · MADE SLOWLY

Dedication

For the adult child standing in a hospital hallway at 11pm,
holding a printed care plan they cannot fully read yet.

You did not sign up for this. You showed up.

A note from Maya

Something has changed.

Your mom had a stroke. Your dad was diagnosed with dementia. Your partner fell. The hospital discharged them with a list of follow-up appointments and a quiet "we'll see." Whichever way it landed in your house, three months from now, the new normal will have been built.

This book is for those three months. The first month is mostly shock and logistics. The second month is the long stretch where the appointment schedule becomes routine and you start to feel the weight. The third month is when the family's pattern stabilizes, for better or for worse.

You are not the patient. You are the person who has been recruited, sometimes without warning, into being the primary witness of someone else's diminishment. That is a real job. It is mostly unpaid. It is rarely acknowledged.

This book is the structure. It does not tell you which decisions to make. It is the place where the questions get organized and the conversations get scripted.

With care, Maya

This is not medical or legal advice. The clinical decisions in your loved one's care belong to them, their medical team, and you and your family acting on their behalf. Financial and legal decisions belong to you, your family, and the financial planner or attorney working with you. The book does not name medications. The crisis-line page is at the back. bookmark it now.

Chapter 1: The Call

The first call comes in some specific way. The hospital. The neighbor. Your father, who never calls, calling.

You take the call. You go.

Then, for the rest of that first week, you make a lot of other calls. To siblings. To work. To your partner. To the loved one's friends. To the insurance. To the discharge planner. To whoever else.

The first chapter is for the calls.

The first call list. Pick a notebook or a notes app and start writing the names down. Primary care provider. The specialist coordinating this. The hospital social worker (often the most important person in the building). The discharge planner. The pharmacy. The loved one's attorney if they have one. The financial planner. The siblings. The partner. The pastor or clergy if relevant. The closest friend who would want to know.

By the end of the first week, you will have called most of these. Some will be useful. Some will be useless. Some will become long-term partners in the care.

Pick a primary contact from the family. This question is going to get asked over and over. The hospital wants one. The doctors want one. The facility, if there is one, wants one. Pick a person. Often the geographically closest, or the one with the most flexibility. It does not have to be the oldest sibling. It does not have to be the one most affected. It does not have to be you, if you can negotiate.

If you are the primary contact by default, the other siblings get to be backup. That negotiation is one of the first conversations of this chapter. The next ninety days are easier if it is settled now.

Chapter 2: Day One

The first day, in your own words. Write it down somewhere. The story of how this started will fade. You will want it later.

The pieces that matter:

- What happened, in plain words.
- Who you have told so far.
- Who you still need to tell.
- What the medical team said today.
- What the next 48 hours look like.
- What you are carrying tonight that you have not told anyone.

The last one is the hardest. Most adult children at the start of a caregiving stretch are carrying things they have not said out loud. Anger at a sibling who is far away. Anger at the parent for not preparing for this. Grief that you cannot fully feel yet because there is no time. Fear that this will go on for years.

Naming the thing, even just in writing for yourself, is the start of holding it without it owning you.

Chapter 3: The First Week

The first week is mostly logistics + shock.

You are calling, driving, sitting in waiting rooms, answering questions, signing things. You may be also working a regular job, which is a separate problem.

What helps:

- Pack a "go bag" for yourself. Phone charger, snacks, a water bottle, paper and pen, a book to read in the waiting room, a sweater for the cold rooms.
- Build a "what they said today" practice. After every appointment or conversation with a provider, write down what they said. The conversations get fuzzy fast.
- Keep one record. Not five notebooks. One place, on your phone or on paper. The team will need this. You will need this.
- Eat. Sleep when you can. Drink water. The basics still matter when the days are intense.

By day seven, you may notice you have absorbed an enormous amount of information. The medical team's names. The terminology of the condition. The schedule of the facility. The personalities of the social workers.

That absorption is real cognitive labor. It is also exhausting. Plan to be wiped out by the weekend.

What is real now that was not real seven days ago. A question to write the answer to, on day seven. The honest answer.

Chapter 4: Weeks 2 and 3

The first month begins to take shape. The shock starts to ease. The work begins to feel like work.

By week two, you may be:

- Settling into a routine of appointments and check-ins.
- Realizing the family workload split is not what you assumed.
- Discovering paperwork you did not know existed (insurance, POA, advance directive, estate).
- Feeling the first wave of resentment toward a sibling who is doing less.
- Crying in the car between things.

All of these are normal.

The paperwork status check. By week three, you want to know where the following stand:

- Durable Power of Attorney (POA), who is the POA, when was it executed, where is the original?
- Healthcare proxy or medical POA, who is named?
- Advance directive or living will, in place?
- HIPAA release for you to receive medical info, in place?
- Will or trust, in place, updated recently?
- Bank accounts, do you have visibility or access (legitimately, through POA if needed)?
- Insurance policies, do you have copies?

- Long-term-care insurance, does the loved one have it, and what does it cover?

If any of these are missing or unclear, the right call is to an elder-law attorney. They specifically handle this kind of situation and are often worth the investment, even if your loved one has a general attorney. Ask the social worker for a referral. Or contact your local bar association's referral service.

Chapter 5: The First Month Closes

Around week four, the new normal begins to be visible.

You may not love it. But it is the new shape.

A question for the end of the first month: what is different now than at week one? Sometimes the answer is "everything."

Sometimes the answer is "I am tired in a way I did not know was possible." Sometimes the answer is "I am more capable than I knew."

Write down a few specifics. The medical team you now know by name. The appointment cadence you have learned. The paperwork you have filed. The siblings who have stepped up. The siblings who have not.

The patient's current functional status. In plain words, not medical. What can your loved one do without help. What do they need help with. What did they used to do that they no longer can.

This list will change over the next two months. Write it down as a baseline. The change becomes visible only when you can compare to something.

Your current role in their care. Primary caregiver. Coordinating but not hands-on. Shared with siblings. Backed by paid help. A mix. Something else.

Whatever you write down for week four will probably be different by week eight. That is the nature of this stretch.

Chapter 6: The Medical Team

By the end of the first month, you have a team. Primary care. The specialist coordinating. Specialist 2, 3, possibly 4 or 5 (cardiology, neurology, oncology, etc., depending on the situation). Hospital team (if currently inpatient): hospitalist, charge nurse, social worker, case manager. Home health, if applicable: visiting nurse, home health aide, PT, OT, speech. Pharmacy. Hospice, if relevant or to consider later.

The team is the team. You are now its coordinator.

A few principles for the relationship:

- Bring written questions to every appointment.
- After every appointment, write down what they said.
- Ask for plain language. "Can you explain that in words I will remember in six hours?"
- Ask for written summaries when something matters.
- If a member of the team is not working for your loved one, ask for a different one. You can switch.

On medications. This book does not name medications. The medical team does, and your pharmacist confirms. Track them by purpose, not by name, in your own notes. The pharmacy keeps the names. You keep the schedule. Who else needs a copy of the current list: a backup caregiver, a sibling involved in care, the loved one's primary care provider, the specialist, the pharmacist, the ER bag (a folded copy in the bag they take to the ER if needed).

A small principle: never write medication brand or generic names in your own workbook. Only the purpose. The medication list is kept by the pharmacy and the provider. Your record is not a secondary medical record and does not need to hold names.

Chapter 7: Financial and Legal Paperwork

The chapter most adult children do not want to deal with. Also the chapter that, if it is dealt with in the first 90 days, makes everything afterward easier.

The documents to confirm or put in place:

- Durable Power of Attorney.
- Healthcare proxy / medical Power of Attorney.
- Advance directive / living will.
- HIPAA release.
- Will or trust.

The financial visibility you want, ideally:

- Bank accounts (their primary).
- Retirement / 401k / pension.
- Social Security status.
- Medicare / Medicaid status.
- Long-term-care insurance (if any).
- Home insurance / mortgage status.
- Last year's tax filing.
- Bills + recurring payments.
- Safety deposit box (location + key).
- Digital accounts (email, social, online banking) and their logins.
- Vehicle title.

You may not have access to all of these. The POA gives you legal authority over many. The will dictates what happens to the rest. The financial planner or accountant, if there is one, holds the rest.

A note: elder-law attorneys specifically handle this type of situation and are often worth the investment. The fee for a few hours of their time is often less than the cost of one mistake made without them.

Chapter 8: Where They Will Live

This is one of the heaviest decisions in the 90 days.

The book does not tell you what to choose.

The options.

- Stay in their own home, with help. Familiar, their stuff, their neighborhood. Logistically intensive. May require home modifications. May not be safe long-term. Costly home-health.
- Move in with you (or another family member). Closer eyes on them. Saves on facility costs. Caregiver burnout. Marriage strain. Loss of their autonomy. Loss of your home being yours.
- Assisted living. Independence for them. Social environment. Some support without medical level. Cost. Adjustment period. Not appropriate for high-medical-needs.
- Skilled nursing facility (long-term care). Medical staff on site. Appropriate for high needs. Cost. Loss of autonomy. Quality varies widely.
- Memory care (for dementia). Specialized for memory disorders. Cost. Often locked unit. Adjustment.
- Hospice (when appropriate). Comfort-focused. Medicare or insurance covered. Care can happen at home or in facility. Requires accepting that prognosis is limited.

What you are weighing. The patient's preference (in their words, if they can tell you). The medical team's recommendation. Your financial reality. What the siblings think (each, separately). What feels right to you, separately from all of the above.

A note. This decision may be revisited. The first choice is not always the final choice. People's needs change. The facilities people go to from home are often a stepping stone, not a final place. Give yourself permission to re-evaluate in a few months.

Chapter 9: The Siblings

Sometimes the family closes ranks. Sometimes the family splinters. Often it does both, on the same week.

The siblings are the part of caregiving that adult children most often underestimate.

The honest assessment.

What you are doing. What sibling 1 is doing. What sibling 2 is doing. What sibling 3 is doing. What no one is doing, that should be.

Write it down. Honestly.

The hard conversation you have not had yet, and with whom. Most caregiving families have at least one of these pending at the four-week mark. The conversation is usually with the sibling who is doing the least, about the workload split. Or with the sibling who is doing too much, about your own role.

The conversation does not get easier by waiting.

Old wounds resurfacing. Family dynamics from when you were younger often come back during eldercare. The favored child. The black sheep. The one who left. The one who stayed. None of those identities get to drive the current decisions, but they all show up in the room.

A family therapist or elder-family mediator may be worth the investment if the conversation cannot move without one. They exist. The social worker can refer you.

Chapter 10: I Am Not Okay Either

Caregiver collapse is well-documented. Caregivers of aging parents have higher rates of depression, anxiety, sleep disorders, marital strain, and their own health issues. The book names this honestly.

The honest check.

How many hours of sleep, average, this past week. How many real meals (not just coffee + snacks). How many days you cried this week. How many days you drank more than usual. How many days you yelled at someone you love. How many appointments of your own you have missed. How many friends you have stopped texting.

How much of your own time is currently yours, in hours per week. Is that enough?

If the numbers are concerning, you are not the first. Many caregivers reach this point at the end of the first month. The treatment is the same: a therapist, a support group, a respite plan, a sibling-level conversation about workload, the willingness to ask the help you have not been asking.

Caregiver crisis lines (US):

- 988, Suicide and Crisis Lifeline.
- 1-800-677-1116, Eldercare Locator (connects to local Area Agency on Aging, caregiver support, respite, more).
- 1-800-445-8106, Family Caregiver Alliance.
- 1-800-272-3900, Alzheimer's Association 24/7 Helpline (for dementia, but they also help with non-Alzheimer's).

UK: 0808 808 7777, Carers UK. 116 123, Samaritans.

Canada: Carers Canada (find local). 9-8-8 Suicide Crisis Helpline.

A reminder: caregiver burnout is a leading cause of nursing-home placement. The data is clear. Taking care of yourself is not selfish. It is what keeps you in this for the long haul.

Chapter 11: What They Want to Remember

An optional chapter. For the legacy work, if you and your loved one have the bandwidth and want to.

If your loved one can still talk with you about their life, this chapter is the place to capture some of it. Not in a "we are documenting them because they are dying" way. In a "I would like to remember this when you are gone" way.

A few questions to ask, one per visit, when the moment is right:

- What is the earliest memory you have?
- Who was the most important person in your life when you were young?
- What is something you are proud of that I might not know about?
- What is a regret you would tell a younger version of yourself about?
- What did you love about your mother / father?
- What did you find hard about them?
- What is one piece of advice you want me to remember after?
- Is there anyone you want to make peace with before you can't?
- What did you wish you had told me, that you never have?
- What do you want at your service, if and when?

A note: you do not have to do this all in one visit. Or any of it. Some people want to do this work. Some do not. Take your cue from your loved one. If they want to talk, take notes. If they do not, do not force it. The legacy work is theirs to consent to.

Chapter 12: The 90-Day Check-in

By the end of week thirteen, you can take stock.

Where the loved one is. Living situation. Functional status. Medical situation. Mental status.

Where you are. On a 1-10 scale. Your marriage / partnership. Your own work / job. Your own health. Your relationship with your siblings.

What is working. Specific things, in plain words.

What is not. Specific things, in plain words.

What needs to change in the next 90 days.

- The care arrangement (where they live, who is helping).
- The family workload split.
- Your own work situation.
- Your own mental health (therapist, group, time off).
- The medical team (different specialist, second opinion).
- The financial / legal paperwork.

A letter to yourself, ninety days from now. What you hope is true. What you are willing to fight for. What you are willing to let go of.

The book closes here. The next book, *The Long Caregiving Stretch*, covers what comes after.

Chapter 13: A Final Letter

You made it through ninety days.

You met the team. You wrangled the paperwork. You sat in waiting rooms. You answered the same questions from siblings five times. You had at least one cry in a parking lot.

The next ninety days will be different. The shock has settled. The patterns are set. The decisions you made may need to be revisited. Some will be wrong. You will make different ones.

What this book cannot do is tell you that everything will be okay. Sometimes it is. Sometimes the loved one stabilizes and a new long chapter begins. Sometimes things continue to decline. Sometimes the loss is closer than the book can address.

What the book can do is hold the structure. The medical team, the paperwork, the conversations, the decisions. So that on whichever day in whichever year you look back, you have a record.

You are doing the work that most of us will do for someone, eventually. There is dignity in it. There is also exhaustion in it. Both are allowed to be true.

You are not alone in this, even when it feels like you are.

With care, Maya

About Soothemade

Soothemade is a small apothecary of printables, planners, cards, and books for new parents and the people taking care of them.

The printable companion to this book is the **Eldercare First 90 Days Planner** at notes.soothemade.com, with day-by-day templates and decision-page forms.

Also from Soothemade

- **The Long Caregiving Stretch**, the next book in the series, for months 4 and beyond.
 - **The Field Guide to Asking for Help**, where the asking muscle gets a workout.
 - **The Family Caregiver Workbook** (printable), the deeper companion for ongoing care.
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Crisis lines

United States

- 988, Suicide and Crisis Lifeline.
- 1-800-677-1116, Eldercare Locator.
- 1-800-445-8106, Family Caregiver Alliance.
- 1-800-272-3900, Alzheimer's Association 24/7 Helpline.
- 1-800-662-4357, SAMHSA National Helpline (substance use).

United Kingdom

- 0808 808 7777, Carers UK.
- 116 123, Samaritans.

Canada

- 9-8-8, Suicide Crisis Helpline.

Australia

- 1800 422 737, Carer Gateway.

This book is for planning and personal use only. It is not medical or legal advice.

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